

pharmaniaga™

Folic Acid

 Tablet

DESCRIPTION

Yellow, round, flat beveled edge tablet with 'KK' marking on one side and breakline on the other.

COMPOSITION

Each tablet contains Folic Acid 5 mg.

ACTIONS

Folic acid is a member of the vitamin-B group. Folic acid is reduced in the body to tetrahydrofolate, which is a co-enzyme for various metabolic process including the synthesis of purine and pyrimidine nucleotides, and hence in the synthesis of DNA; it is also involved in some amino-acid conversion, and in the formation and utilisation of formate.

Without folic acid, the living cell cannot divide but is halted in metaphase. This property underlies the use of folic acid antagonists in the treatment of neoplastic disease. Folic acid is necessary for the normal production of red blood cells, including maturation of megablasts into normoblasts.

a) Absorption:

Folic acid is rapidly absorbed after oral administration, mainly from the proximal part of the small intestine; absorption is greatly reduced in coeliac disease; anticonvulsants may inhibit the absorption of folate.

b) Blood concentrations:

Physiological blood concentrations are about 350 mg/ mL.

c) Half-life:

Biological half-life is about 0.7 hours.

d) Distribution:

Widely and rapidly distributed throughout the body and large amounts are stored in the liver, mainly as methylated folate; normal body stores are of the order 5 to 10 mg, 50% of which is found in the liver; protein-binding, about 66% bound to plasma proteins.

e) Metabolic reactions:

Metabolized by reduction and methylation to 5-methyltetrahydrofolate and by oxidation to folinic acid.

f) Excretion:

20 – 90% of an ingested dose is excreted in the urine in 24 hours; the percentage of a dose excreted appears to be dose-dependent as after a 5 mg dose, 50% is excreted in 24 hours and after a 15 mg dose about 90% is excreted in 24 hours; urinary excretion products include folinic acid; excretion is increased after the administration of methotrexate.

INDICATIONS

For the prevention and treatment of folate deficiency states as may occur in macrocytic anaemia, megaloblastic anaemia of infancy and pregnancy, pellagra, sprue, idiopathic steatorrhea, coeliac disease and following gastrectomy.

CONTRAINDICATION

Folic acid should not be given alone or with insufficient amount of hydroxycobalamin for the treatment of megaloblastic anaemia because through folic acid produces a haemopoietic response it fails to prevent the onset of subacute combined degeneration of the cord.

ADVERSE REACTIONS

Folic acid is usually well tolerated.

PRECAUTIONS

The inclusion of folic acid in the multivitamin preparations may be dangerous. Large doses of folic acid may lower the blood concentration of vitamin B12. Caution when administering folic acid to patient who have folate dependent tumours and to epileptics (lower plasma concentration of phenytoin occasionally). Folic acid should not be given until the diagnosis of pernicious anaemia has been ruled out.

Pregnancy and Lactation

None.

DOSAGE AND ADMINISTRATION

Adult:

Megaloblastic anaemia: 10 mg to 20 mg daily oral dose for 14 days or until haemopoietic response has been obtained. Daily maintenance dosage: 2.5 mg to 10 mg.

Children:

May be given maintenance doses daily according to the following regime:

Up to 1 year – 250 mcg / kg / day

1 to 5 years – 2.5 mg / day

6 to 12 years – 5 mg / day

(An initial dosage of twice the maintenance dose can be given for two days)

ROUTE OF ADMINISTRATION

Oral

OVERDOSAGE:

Symptoms and Treatment

Oral overdosage with folic acid is quite unlikely.

STORAGE CONDITIONS

Store below 30°C. Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In blisters of 6930 tablets.

Date of revision: 11th November 2016

Product Registartion Holder/ Manufacturer:

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